

UK Recall 2025

- * **History**: short stature 10 years old
- * **Develop**: $2\frac{1}{2}$ child for Cognitive $\bar{e}$ autism and hearing impairment
- * **Com 1**: Constipation $\bar{e}$ soiling Concerns regards specific duration of ttt need to give exactly 8 ttt of soiling.
- * **Com 2**: Trisomy refused genetic referral Tell her the importance of genetic referral & mgt plan.
- * **Video 1**: scarlet fever
- * **Video 2**: arrested epileptic pt after diazepam
- * **Clinical**:
 - spina bifida
 - C.N examination
 - spherocytosis
 - eye exam in pt $\bar{e}$ thyroid scar
 - pansystolic murmur
 - pt $\bar{e}$ AS

Malasia Day 1 Feb 2025

* **Video**: Bell's palsy
ITP

* **Develop**: William syndrome & ADHD
speech & social

* **Clinical**:
- Ataxic gait, cerebellar exam
- Midline sternotomy & mitral click → valve replacement
- chronic supp lung disease
- Glycogen storage disease

* **Hx**: SLE

* **Com**:
1. TB diagnosis & mgt
2. SVT

Malaysia Day 2 Feb 2025

* **Clinical**: CVS → 11 y intermittent chest pain
No scar, pansystolic murmur radiate to
Lt Axilla



CNS: 8 years UMNL L.L → hyperreflexia
spine normal, orthosis, can't walk

Msk: 11 y small, multiple hypopigmented
lesions on extensor surfaces
limited flexion & extension of wrist
No rashes

other: 11 y dysmorphic, eyelid capillaries
high arched palate, small mouth
No jaundice, roof top scar in abdomen
Liver 2cm spleen 7cm

*Com: 1. small VSD & bad dental care
developed fever wbc's + CRP ↑↑
Culture → strep viridans
explain IE to father
Can it be prevented, My fault?
How long antibiotics?
Busy father leave kid & care giver
so offer solution.

2. 8 y Type 1 DM & since 3 years ago
not well controlled in last 1 year
HbA1c 10, 3 DKA, 6 months ago
& UTi

child don't know how to inject. Mom has other kids to take care of. want to change 3 dose to 2 dose injection ask what DKA? will it affect school?

* **History:** 1y prolonged seizure, fever & URTI 3 days before. Admitted to ward done baseline invest all normal had brief seizure since 3 months old Multiple seizure med latest on syrup Keppra & vigabatrin only + GDD Compliant to ttt. Done brain MRI +ve Consanguinity. 1st child passed away at 2y & lung infection & developmental delay.

* **Develop:** 4y ex preterm 35 wks Bw 2kg Hx of bowel operation d3 of life with brief Cardiac arrest. admitted total 1 month, ventilated 4 days MRI at 6m show periventricular leukomalacia (white matter problem) had seizure after that, not on any meds, No feeding issue
→ spastic diplegic Cp due to HIE
GDD

* **Video:** 1. Nephrotic syndrome
2. Asthma

Recent recall Feb 2025 (OMan) D2

* **Video:** Kawasaki,
GBs
Mastoiditis

* **Com:** - Febrile neutropenia (wilm's)
doesn't want to stay in hosp.

- angry mom no seat in hospital
for neonatal Jaundice

uncotrolled Asthma - ethical dilemma ^{AI hepatitis}

* **History:** ketatic hypoglycemia

* **Develop:** Hemiplegia, fine motor

* **Clinical:** - wheel chair UMNL (diplegia)
- Cranial Nerve exam
- CMT
- Ps, VSD, Noonan

- chronic suppurative lung dis
- Chest deformity
- immunodeficiency
- sickle cell anemia.

Jeddah Feb 2025 Day 1

* **History**: Hsp → non blanchable rash, abdominal pain, bilateral ankle arthritis.

* **video**: 1. acute severe asthma
mgt plan + doses

2. leukemia → cervical L.N,
Hsplenomegaly, rash

* **Com 1**: obese & chest infection diagnosed
Type 2 DM, high RBs & HbA1c
BMI 35, all family obese, Mom
is angry as Gp did labs without
telling her, Talk about mgt plan

* **Com 2**: Newborn few hours 35+ wks
PROM 28h, plan to start I.V benzy



+ gentamicin and mother refuse

* **Development:** Hemiplegia motor
- birth asphyxia, NICU admission
for 2wks, seizure in 1st day of
life, discussion about head
cooling.

Jeddah Feb 2025 Day 2

* **Com:** 1. TB $\bar{e}$ +ve NAT
2. Celiac

* **video:** - ITP
- facial balsy

* **Hx:** SLE

* **Develop:** abnormal behavior william

* **clinical:** DextroCardia
Thalassemia
hemiplegia.
ps
chronic supp lung

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OMan Feb 2025 Day 1

* **History**: kawaski → fever 7 days,
rash, abd pain

* **Com 1**: wrong bottle feeding

* **Com 2**: CF related DM

* **Video**:
- pertussis
- Chorea

* **Clinical**:
- Chronic suppurative lung
- UMN
- VSD

* **Development**: Cognitive

Mumbai Feb 2025 Day 1

* **Clinical** : - chronic suppurative lung.
- MR
- Thalassemia $\bar{e}$ HsM
- L.L motor exam

* **Com** : - NAI Counsel the mother
- Counsel 15 y old for H. pylori treatment

* **Hx** : poor wt gain & anemia

* **Develop** : Neuro regression $\rightarrow$ gross motor

* **Video** : - Erb's palsy
- stridor

Mumbai Feb 2025 Day 2

* **video**: 1. SSSS rash in 5 months old
ē peeling of skin

2. PDA in preterm.

* **clinical**: - L.L motor ē knee Contractures
- Chronic supp lung
- thalassemia
- MR ē cyanosis

* **Development**: speech delay → Cognitive

* **Hx**: 1st episode of unprovoked seizure (GTCS) in a known case of febrile seizure. father worried about tumor

* **Com 1**: RSV infection in hospital

* **Com 2**: preterm ē NEC & TPN needs to be started on enteral feeds

Egypt Feb 2025 Day 1

*Com 1: Ethical dilemma to med student about limited resource. one baby ē multiple organ failure other is good preterm.

*Com 2: Nocturnal euresis

*video: Hsp
Laryngeomalacia

*History: Leukemia (ALL)

*Develop: Neuroregression ē fits
For Cognitive - DMD ē autism

*clinical: Thyroid
Asthma
diplegia
short stature
NF
MR, VSD
Achondroplasia

Egypt Feb 2025 Day 2

* Video: 1. stridor → Croup in 3 months

2. EBV → blanchable rash

* Com: 1. intussusception & role of Xray and how to relieve it

2. subarachnoid Hge needs phenytoin

* Clinical: MPS
Kartagener
Hemiplegia
Diplegia
Thalassemia
MR → RF details
Thyroid

* Hx: 9 months baby noted in nursery has squint, went for Gp white spots in his eye. All Hx free. Develop normal DD of squint → Retinoblastoma



*Com 1: Teenager subarachnoid Hge after head trauma got Comatosed and ventilated Now recovered
Talk to her about what happened
* going trip & friends

*Com 2: Intussception

*Develop: Autism

Hyderabad Jan 2025 Day 1

*video: - Erb's palsy
- Mastoiditis

*Com: 1. cleft lip & palat
2. VZV prophylaxis for Nephrotic on steroid exposed to chicken pox in hospital.

*HX: Bone tumor

*Develop: Down & recurrent OM
for Cognitive assessment



- * **Clinical** :
- Thalassemia
 - hemiplegia
 - Turner short stature
 - VSD, MR.

Hyderabad Jan 2025 Day 2

- * **Clinical** :
- Msk short stature → alopecia + eye glasses
 - spastic hemiplegia
 - MR ē RHD
 - Thalassemia, H. spherocytosis

* **Develop** : fetal alcohol, fine motor

* **HX** : Infan ē seizure → hypocalcemia

* **Com 1** : - vulvovaginitis
- Croup ē extra dexta dose

* **Video** : - dermatitis artifacta
- varicella ē 2ry bact infection

Jeddah Jan 2025 Day 1

*Video : 1. pertussis
2. post viral cerebellitis

*Com : 1. WPW
2. familial short stature

*Develop : 4 y DMD crawling on stairs & frequent falling and abnormal behavior

*Clinical :
- Thalassemia
- Residual ps
- Treacher Collins
- hemiplegia
- LMNL & renal fistula
- short stature (general)
- Quadriplegic

*Hx : 9 months boy, mother smoker during pregnancy, SGA, forceps delivery, Jaundice, low platelet microcephaly, Bruises diagnosed CMV, develop delay, squint, vision and hearing problem (TORCh)

Jeddah Jan 2025 Day 2

- * **Clinical**:
- Rt hemiplegia
 - CP & CVS & pacemaker
 - Thalassemia Jaundice HSM
 - Kartagener: hearing aid, Dextrocardia, clubbing
 - Residual ps (ToF)
 - Msk: hypermobility ED
 - Klippel Feil syndrome
 - Quadriplegic CP

- * **Video**:
1. focal seizure 2 months
Could it be late onset Hgic dis of newborn?
 2. Croup

- * **Develop**: 4y Fine & milestone
Large head, thick hair, Hx of febrile seizure which stopped DM during pregnancy, feeding issue & the baby. obese and hypoglycemia was there
Could it be Soto?

* **Hx**: Celiac → diarrhea, fTT, DD of malabsorption

* **Com 1**: Fracture humerus

* **Com 2**: - Functional abdominal pain
mother worried about ulcerative colitis as she was diagnosed ē it.

Egypt Dec 2024 Day 1

* **clinical**: Mosaic turner
ps
diplegia
Thalassemia
GSD

* **Hx**: DM ē loose motion & wt loss

* **Develop**: GDD ē autism (cognitive)

* **Com**: 1. child not gaining wt, concern about CMPA
2. Asthma non compliant

* **Video**:
1. Congenital pneumonia
2. Dermatitis artifacta

* **others**:
Cushing → effect on eye
achondroplasia
MR post operative
Williams

Egypt Dec 2024 Day 2

* **Hx**: difficult walking & speech (DMD)

* **Develop**: Mps ē fine motor

* **Com**:
1. challenge behavior want
meds for ADHD
2. Baby on photo therapy has
dehydration.

* **Video**:
1. Croup
2. eye swelling anaphylaxis

* **Clinical**: same day 1
- post Kasai ē hepatomegally

Jeddah Nov 2024 Day 1

* **Develop**: 2.5 y GDD
microcephaly ē epilepsy
Motor & milestones

* **Hx**: 12y chest pain functional due to social issues

* **Com**:
1. PDA
2. Tension Headache

* **Video**:
1. Croup
2. pulled elbow

* **Clinical**: Quadriplegic CP
Chronic kidney disease
Noonan short
Dextrocardia ē murmur on pulmonary area & scar.

Jeddah Nov 2024 Day 2

* **Hx** : Headache for DD
(brain tumor)

* **Develop** : Recurrent ear infection for
Cognitive

* **Video** :
1. SSSS
2. Reactive arthritis
(Hx of 3 days)

* **Com** :
1. aminophyllin wrong dose
2. ABO incompatibility &
exchange transfusion.

* **Clinical** : UMNL bedridden quadriplegic
hemiplegia
Ehler danlos
Marfan
Noonan
residual ps
pacemaker
Thalassemia

Pakistan Nov 2024 Day 1

* **Com:** 1. Familial short stature
adolescent want ttt

2. Hypertrophic pyloric stenosis
ē deranged ABG
parents wanted surgery rt now

* **Video:** 1. peri orbital Cellulitis
2. NAIT

* **Hx:** epilepsy difficult to Control
presented ē status epilepticus

* **Develop:** Sturge weber

* **clinical:** DMD
hemiplegia
Down ē resp exam (OSA)
Rickets
USD
GBs (AFP)
GSD
hepatosplenomegaly

Pakistan NOV 2024 Day 2

- * **Clinical**:
- short stature + knock knee (look at legs & proceed)
 - Hemiplegia (both ul, L-L)
 - Chronic supp lung & lt lobectomy scar & chest deformity
 - look at gait (short, knock knees, scoliosis, chest deformity)

- * **Com**:
1. Audit of ORs & Colleague
 2. Autoimmune thyroiditis deranged TSH, T₄. Talk to mom for further Labs & evaluation has also loose stool.

- * **Video**:
1. pneumonia (resp exam of back)
 2. Osteomyelitis (lt leg swelling)

- * **Hx**: Short stature. His aunty has Concern and he is adopted child
→ GH def or familial short stature

* **Develop:** GDD 2 years do fine motor (IEM, Torch, Kernicterus)

Nepal Nov 2024 Day 1

* **Video:** 1. DKA d.d, ttt, Comp
2. Bell's palsy

* **Clinical:**

- Diplegic cp
- Hemiplegic cp
- ASD
- Thalassemia
- Down General

* **Develop:** diplegic cp Gross motor

* **Hx:** Hematuria Hsp & nephritis

* **Com:** 1. Soya milk talk to mother wanted to give cereals

2. NAI 4 months Came & bronchiolitis, 2 bruises on Rt maxilla, 1 on lt maxilla
Talk about mgt

Nepal Nov 2024 Day 2

- * **Com:** 1. Neonate ē hypoglycemic seizure
2. Nephrotic syndrome
- * **Hx:** Relaps of Crohn's
- * **Develop:** DMD gross motor
- * **Clinical:** Quadriplegic cp
VSD, dextrocardia
Thalassemia ē splenectomy
- * **video:** 1. Lymphadenopathy
2. Mastoiditis

UAE NOV 2024 Day 1

- * **Hx:** Cf * **Develop:** DMD
- * **Com:** 1. Neonatal seizure following hypoglycemia
2. N.S mother refuse steroid
- * **video:** 1. pyloric stenosis 2. mastoiditis

UAE Nov 2024 Day 2

*Hx: pneumonia e SIADA

*Com: 1. Constipation to mother

2. ADHD e abnormal behavior
Teacher concern

*video: - ITP
- Arthritis (Reactive)
(Transient synovitis)

*Develop: - speech and social in child
admitted to baby unit
large head. GDD

*clinical: - Residual ps
- Cf for follow up
- hemiplegia (UL)

UK October 2024

* **video**: 1. infantile spasm
2. viral croup

* **Com 1**: 13y has anorexia nervosa. CAMHS involved in her mgt lost 2 kg in last month has bradycardia & hypothermia. Talk to mother about mgt plan, nutritional support blood sugar 3 ketones +4

* **Com 2**: NAI 9month, bruises on thigh and cheeks, was hit by her partner. Mother is anxious and worried about her son.

* **Hx**: adopted 6 months with frequent vomiting.

* **Develop**: 2y child of 17 years mom fine motor assessment

* **clinical**: EDs, Marfan, poland residual murmur ē median sternotomy scar

UK October 2024

* **Hx**: anaphylaxis, Mom ē asthma, brother ē allergy

* **Com 1**: 14y girl, paracetamol toxicity & self harm, Lft normal has unprotective sex and worried about STDs

* **Com 2**: Busy mother, her son asthma having frequent attacks, not taking brown inhaler, father is smoker.

* **Develop**: Motor, GDD, cataract, Consanguinity, his uncle has mobility issue

* **video**:
1. 10 months ITp
2. 12y 5 days vomiting → 6 nerve palsy

* **Clinical**:
10y ps no scar
9y Rt hemiplegia
11y Jaundice abdomen
9y turner short ē transverse scar in the abdomen

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Jeddah sept 2024 Day 2

* **Com** : 1. Hsp
2. All

* **video** : 1. shingles
2. facial palsy

* **Hx** : Down ē hypothyroidism &
Constipation.

* **Develop** : sturge weber → Cognitive
& milestones

* **clinical** : H. block ē pacemaker
Quadriplegic
EDs
Klippel Feil
Thalassemia
Renal failure
Complex Congenital HD
Kartagener
Short stature

UK October 2024

- * **Development**: 2y & 3 months, fine motor, Hx of febrile fits
- * **Hx**: 6y $\bar{e}$ syncope, needed O₂ in ER
→ myocarditis.
- * **Com 1**: 16 wks sweat test & gene test +ve
have loose stool. asked about school & the condition.
- * **Com 2**: 16y paracetamol 2 packs after 4h PCM level 240 Lft normal to be discharged. Mom refuse to talk to CAMHS
- * **Video**:
 1. meningococcal rash
 2. Nephrotic syndrome
- * **Clinical**: 16y arachnodactyly, pansys murmur radiate to Axilla
 - LMNL short obese on wheelchair
 - 3y vertical abd scar $\bar{e}$ peg tube
 - Msk Normal UL PGALS



UK October 2024

- * **Com 1**: Type 1 DM came $\bar{e}$ seizures
talk about plan & mgmt &
recurrent hypoglycemia
- * **Com 2**: Talk to mother of newborn $\bar{e}$
Jaundice who you decided to
transfer due to non availability
of beds
- * **video**:
 1. pleural effusion
 2. celiac disease
- * **Hx**: 5 months with excessive cry and
bruising $\rightarrow$ NAI
- * **Develop**: 2y $\bar{e}$ infantile spasm and
neuroregression $\rightarrow$ Motor
- * **clinical**:
 - 16y $\bar{e}$ median sternotomy scar
 - 14y $\bar{e}$ ptosis examine face
and cranial nerves
 - EDs
 - Adolescent $\bar{e}$ liver transplant
scar.

UK October 2024

* **Hx**: 5y ē recurrent UTI

* **Develop**: 4y brain tumor ē chemo
& radio → assess fine motor

* **Com 1**: 6wks ē laryngomalacia
mother insist on admission

* **Com 2**: Explain to grandmother who
is arheumatologist regarding her
grandson born preterm require
surfactant

* **Video**: 1. Infantile spasm
2. ITP

* **Clinical**: Quadriplegic, hemiplegic cp
11y abd epigastric scar
and peg
NF
Resp: FTT, short, tracheostomy
scar
Down ē VSD
Cf
Abd: hepatomegaly, ascites
distended vessels, stunted
growth

Singapore october 2024

* **Develop**: 2y regression, Rett syndrome

* **Com 1**: MRSA Colonization in NICU
ex preterm, discuss the result

* **Com 2**: 2y aggressive behavior mom
think it's ADHD want to start
methyl phenidate

* **Hx**: syncope

* **Clinical**: Abd → liver transplant scar
ē portal hypertension & jaundice
Resp → 2y Resp distress,
Crepitation & wheezing for DD
- Hemiplegia
- Residual ps

Jeddah Sept 2024 Day 1

* **Com 1**: Newly found pancytopenia
Counsel mom for further
workup

* **Com 2**: Teenager ē peanut allergy
not serious enough to carry
epipen and suffered from
anaphylactic attack at party

* **Hx**: Celiac disease (FIT)

* **video**: 1. GBs
2. Laryngomalacia

* **Clinical**: Rt hemiplegia
Thalassemia
Noonan
Kartagener

* **Development**: Fragile x for social
and speech ē Autism

Egypt sept 2024 Day 1

- * **Develop**: Down, Constipation, obese
Thyroid, GDD 4y →
Fine motor
- * **Hx**: anaphylaxis, has eczema
also, mother has asthma
- * **Com 1**: It's not appendicitis
it's UTI.
- * **Com 2**: DMSA 30% scar
- * **video**: 1. DDH
2. Asthma
- * **clinical**: GSD
short stature
waddling gait & myopathy
For D.D

Egypt sept 2024 Day 2

* Com: 1. Gastrostomy
2. Infantile spasm

* Develop: shaken baby & hemiplegia

* Hx: chronic asthma

* Video: 1. Cushing
2. Mastoiditis

* clinical: LMNL myopathy?
Residual murmur
Noonan & scar
Thalassemia

UK June 2024

* **Com 1**: 15 y ē anorexia nervosa

* **Com 2**: 15 months admitted by URTI
started AB after that radiology
said its thymus. Talk to mom.

* **video**: 1. PDA : pre term ē
machinary murmur
2. 37 wks distressed →
Congenital pneumonia

* **Hx**: Child ē fever 3wks, diarrhea
wt loss came from india (HUS)

* **Develop**: 3 $\frac{1}{2}$ y, Somali refugee
under foster care for 6 months
do social & speech
has tantrum & hearing problem.

* **Clinical**: 5 y ē post Kasai
quadriplegic CP
Turner
syndromic ē tracheostomy
tube O₂ dependent.

Egypt June 2024

* Hx: Functional constipation
due to feeding habits

* Com: 1 EBM

* Com 2: Rolandic epilepsy

UK June 2024

* **Hx**: GE ē blood in stool

* **Develop**: 2.5 y ē behavioral and hearing issues, now in foster care, Biological mom is alcoholic and used Cannabis → gross motor

* **video**:
1. Lt knee swelling
2. Mastoiditis

* **Com 1**: mother on detoxification regimen. mgt of newborn

* **Com 2**: Talk to mother for admission of 13y anorexia nervosa

* **Clinical**: Noonan ē ps
NF
dysmorphic, chronic lung
on nasal oz do general.

UK June 2024

*Hx: 8y ē CF, 2wks Cough, wt loss
sputum Color yellow green

*Develop: 3.5y delayed writing ē
hyperactivity
also gross motor delayed
Assess fine motor.

*Com 1: mom diagnosed ē gluten
insensitivity enteropathy
Talk to Lucy teenager about
this.

*Com 2: 4y Nephrotic syndrome ē
4 relapses tried Levamisole
but no response
After spent weekend ē father
Came edematous protein 3+
Cr 115 Bp 110/75
talk to mom about mgt

*Video: 1. mastoiditis

* **Clinical** : Marfan
Cushing in chest case

Singapore June 2024

* **Hx** : simple obesity & OSA on over night Bipap with worsening difficulty in breathing.

* **Develop** : 2.5y Down, recurrent ear infection, GDD
→ gross motor

* **Com 1** : wrong bottle feed instead of breast milk. Mother has eczema.

* **Com 2** : Rt dilated renal pelvis 16mm
talk about need to start prophylactic AB and do u/s within a week

* **video** : 1. shingles : 6 months on oral steroid
2. GBs : 10 days symptoms

* **clinical** : thalassemia
repaired spinal dysraphism
Tof ē residual ps
Eye examination.

* **UK June 2024**

* **Develop**: 3.5y Fetal alcohol . Hx from
grand mother → Motor

* **Hx** : Diabetes in Down with hyperglycemia
and lipodystrophy , Needle phobia

* **Clinical** : - Residual ps
- Jaundice, pallor splenomegaly
- Hypotonia on wheel chair
ē peg & catheter, Kyphosis
- Turner short stature

* **Com 1** : Asthmatic child needs to start
brown inhaler and father smoker
use blue inhaler alot

* **Com 2** : chronic fatigue syndrome
to teenager

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* Video 1: shingles

* Video 2: Bilateral eye edema
(hereditary angioedema)

UK June 2024

* Clinical: spastic diplegia
post renal transplant
Syndromic & recurrent chest
Murmur max at Aortic

* Develop: $3\frac{1}{2}$ under foster care →
social & speech.

* Hx: 6y Fuo 3wks, lethargy, refugee
from india, probably infective

* Com1: Anorexia nervosa BMI 16.5
talk about mgt

* Com2: Thymus wrongly diagnosed
as pneumonia on chest xray

uk June 2024

- * **clinical**: spina bifida
EDs
VSD non operated
achondroplasia
- * **Develop**: fetal alcohol → motor
- * **Hx**: Breath holding attack
- * **video**:
 1. Erb's palsy
 2. unilateral Cervical lymphadenitis
- * **Com 1**: Teenager ē Cf ē pseudomonas
refuse Ab and admission
- * **Com 2**: child admitted 2 wks ago
ē eczema herpitaum
Came today ē moderate eczema
referred by Gp

UK June 2024

* **Com** : 1. Rop
2. microcephaly & GDD

* **Hx** : hyperthyroidism

* **video** : 1. Gilbreth
2. Croup

* **Develop** : Bechwith widman → speech and social.

* **Clinical** : hemiplegic CP
NF
Residual ps
Liver transplant

Egypt June 2024 Day 1

* **Com**: 1. EBM
2. girl ē rolandic epilepsy

* **Video**: 1. CcHD
2. SSSS why not EH

* **clinical**: DMD
ps
Thalassemia
short stature → mosaic
turner, Noonan

* **develop**: Fine motor GDD

* **Hx**: Constipation

Egypt June 2024 Day 2

* **clinical**: UMNL Quadriplegia
diplegia
ps, MR
kartagener
Arthritis

* **Com**: 1. Rop
2. GDD é microcephaly.

* **Develop**: Beckwith widman →
Cognitive

* **video**: 1. Jaundic DD
2. Croup

* **Hx**: Celiac é hyperthyroidism.

Jeddah may 2024 Day 1, 2

* **clinical**: EDs
Thalassemia
UMNL spastic gait
residual Ps
Treacher Collin
Kartagener
Russel silver
Rt hemiplegia
MR or VSD

- * **Com**:
1. Huntington Chorea
 2. Jaundice ē medical student
 3. 24 wks just born no available NICU need to transfered 100km away To tertiary hospital.
 4. 24h dilated renal pelvis needs prophylactic Ab and Uls after wk
 - 5- glycopyrolate to cp child ē recurrent chest infection & Chocking
 - 6- Circumcision.

- * **video**:
1. arthritis (JIA) / SLE
 2. post viral cerebellitis
 3. Intestinal obstruction
 4. Kawasaki
 - 5- ITP
 - 6- pleural effusion

- * **Hx**:
- Cf went on trip Came ē Cough
 - Crohn's ē relapse or non Compliance
 - Tension Headache

- * **Develop**: - Autism ē GDD
- DMD
- William 3 years fine motor

Pakistan May 2024 Day 1

* **Hx**: Headache

* **Com 1**: Jaundice ē medical student

* **Com 2**: Huntington disease

* **Video**:
1. JIA
2. pneumonia ē pleural eff

* **clinical**:
GSD
Asthma
Cp
short stature, rickets

Pakistan May 2024 Day 2

- * **Develop**: William, Concern about behavior, ADHD, GDD
Social assessment
- * **Hx**: 9 months failure to thrive
rash, loose stool after weaning
Celiac / CMPA
- * **video**: 1. Blanchable rash mostly JIA
2. epilepsy syndrome (Lennox)
- * **Com 1**: suggestive CoA
- * **Com 2**: Obesity to medical student
- * **clinical**: Asthma
As → murmur at Rt CSE
Diplegic Cp
Hepatosplenomegaly

UAE May 2024 Day 1

* **Develop** : 2.5 y for speech & Language
GDD + recurrent ear inf

* **Hx** : Maturity onset DM

* **Com** :
1. Anorexia nervosa talk to mother
2. Medical student Cf newborn screening

* **video** :
1. post varicella cerebellitis
2. SSSS

* **Clinical** :
spina bifida
Mps + HsM
AR
skeletal dysplasia

UAE May 2024 Day 2

* **Develop**: Cognitive in 3.5 y Autistic

* **video**: 1. acute viral hepatitis
2. pleural effusion.

* **Com**: 1. Vomiting in 3wk baby talk
To medical student mainly
GERD

2. Febrile Convulsions 4th time
Mother needs workup
Talk to her

* **Hx**: Day time enuresis & Constipation
and social issues

* **clinical**: CF
Diplegia bedridden
Thalassemia
VSD



Malaysia May 2024

* **Clinical** skeletal dysplasia
Bronchiectasis
AsD
diplegic cp

* **Com 1**: Talk to midwife about admitting
baby ē Neonatal jaundice and
10% wt loss

* **Com 2**: Talk to mother whose child
has T1DM Came ē DKA
Mgt and sick day roles

* **Com 3**: ADHD & Ritalin

* **Com 4**: UTI Uls normal Counsel
mother for follow up &
prophylaxis

* **Video**:
1. pertussis
2. ITP
3. Bell's palsy
4. SSSS

* **Hx:** orbital Cellulitis

stiffness during feeding (GERD)

* **Develop:** Tall & macrocephaly
Fine motor assessment

speech and social, Autism
regression & new onset of
seizure

Egypt May 2024 Day 1

* **Clinical**: ps
Ataxia
spherocytosis / thalassemia
Thyroid

* **Develop**: Rett syndrome & Autism

* **Hx**: anaphylaxis

* **Com**: 1. thalassemia 17y about
puberty, pregnancy

2. Mother of 2-3 y child
Ecoli, multiple venupuncture
mom is angry

* **video**: 1. Bronchiolitis
2. Dermatitis artifacta

Egypt May 2024 Day 2

*Video: 1. pneumonia & effusion
2. Erb's palsy.

*Com: 1. Eczema don't want referral
to Gp

2. Medical Colleague don't follow
The guidelines

*Hx: relapsed Crohn's / ulcerative colitis

*Develop: NAI for motor assessment

*Clinical: ToF & residual ps
MR
SMA3
Hemiplegia
Quadriplegia
GSD
JIA.

March 2024

* **Clinical**: Quadriplegia
Treacher Collins
operated USD
operated ps
EDs

* **Hx**: JIA

* **video**: Facial Nerve palsy
Hsp

* **Development**: Foster mother, GDD

* **Com**: 1. Lumbar puncture apology

2. leukemia & fever (febrile
neutropenia)

OMan March 2024

- * **Clinical**: Rt hemiplegia
CF
Residual ps
splenomegaly
- * **Develop**: Gross motor
CMT / DMD
- * **Hx**: Asthma
- * **Com 1**: pyloric stenosis and fluid
mgt to medical student
- * **Com 2**: Foster baby detected to have
Tof → talk to social worker
- * **video**: 1. Meningiococemia
2. ophthalmia neonatorum

Jeddah march 2024 Day 1

*Clinical: Thalassemia
Quadriplegic
Treacher collin
operated heart ē ps

*Com: Talk to mother ē Leukemic
child Came ē fever

*Video : 1. Hsp
2. Laryngeomalacia.

UAE NOV 2023 Day 2

*video: 1. facial palsy
2. Exacerbation of asthma

*Com1: NAI ē fracture humerous
Mom works lives ē new partner

*Com2: Controlled epileptic 15y girl
had occulogyric Crisis

*Develop: 2y pVL GDD diplegia

*clinical: Allagile syndrome
diplegia in Down
Mps for Cvs exam
Thyroid exam

*Hx: Down syndrome ē ALL 4 years
adopted child

*clinical: pacemaker
VSD or MR
Rickets ē portacath
spina bifida
Thalassemia ē splenectomy

UAE Nov 2023 Day 1

* **Clinical**: Rt side hemiplegia

pansystolic murmur, PS

chronic hemolytic anemia

Achondroplasia

Mps

* **Hx**: addison disease crisis

* **Develop**: Fine motor GDD 2 y
Regression.

* **Com**: 1. Medical error Dexa 10 dose
in croup
2. growing pain

* **Video**: 1. Infant $\bar{e}$ tracheostomy
& distress
2. Bell's palsy

Jeddah Jan 2024 Day 1

* **clinical**: HSM, Laprotomy scar
Thalassemia
short stature (Noonan,
Russel silver)
Hemiplegia
Quadriplegia
Dextrocardia Kartagener
PS
MSK hypermobility

* **Com 1**: Tracheo esophageal fistula

* **Com 2**: Toddler diarrhea.

* **Hx**: arthritis in Hsp

* **video**: 1. unilateral high steppage gait
2. pertussis

* **Develop**: Motor
at 4 month Concussion, Ts
and regression after that

Jeddah Jan 2024 Day 2

* **Com**: 1. vit k
2. Resume milk in NEC

* **Hx**: epilepsy

* **Develop**: 18 month fine motor

* **video**: self harm
Mono arthritis

* **Clinical**: ps
Thalassemia
Quadriplegia
Treacher collin

* **Hx**: Nephrotic $\bar{e}$ high Bp

* **develop**: Down $\bar{e}$ hypothyroidism

* **video**: pertussis
post varicella cerebellitis

* **Com**: growing pain
NAI

India Nov 2023

* Video: 1. pertussis
2. post ~~p~~ varicella cerebellitis

* Hx: Hx of nephrotic syndrome
with high blood pressure

* Develop: 4 y Down have regression
due to Congenital hypothyroidism
Do fine motor

* Com1: talk to mother whose child
came to ER, other 2 children
came to ER in last 6 months
Talk about safety measures

* Com2: Growing pain

* clinical: VSD
diplegic CP
Thalassemia
general short stature

Nepal sept 2023

* **Clinical**: Rt hemiplegia
VSD
General Down → ē Lat
thoracotomy scar
Thalassemia

* **Develop**: Macrocephaly, delayed
walking → do motor

* **Hx**: IDDM ē uncontrolled blood
sugar

* **Com**: 15 year talk about H. pylori
mgt

* **Com2**: Dexa 10 doses in croup

* **Video 1**: 37 wk grunting ē
tachypnea.

* **Video 2**: ITP

India October 2023

* **Clinical**: Thalassaemia
spina bifida
Mixed cp
VSD

* **Hx**: Multiple food allergy ē
anaphylaxis + asthma in 7Y

* **Develop**: speech and language
isolated delay, snoring
recurrent ear infection
Expressive delayed than
receptive

* **Com 1**: 3 days baby ē fit uls
Rt cerebral ischemia
talk to mother

* **Com 2**: wrong lp to mother

* **Video 1** GBs

* **Video 2** Croup

Nepal sept 2023

* Com 1: Nephrotic syndrome & relapse

* Com 2: familial short stature
Mother needs investigations

* Hx: 3 days fever, 3 episodes of
infections require 3 Ab Courses

* Develop: 18 months, grandmother,
small head, Traumatic delivery
Do motor → GDD

* Video: Leukemia
Bell's palsy

* Clinical: VSD
hemiplegia
General Cong hypothyroidism
Thalassemia
spina bifida
MR in marfan



*Com Hsp ē urine blood and protein
mom worried about
meningococemia

*Com Fracture humerus Talk to
angry mom

*video : asthma exacerbation
Facial nerve palsy

Sept Egypt 2023 Day 1

* **video** : croup
scarlet fever

* **Hx** : 8 months $\bar{e}$ fits due to hypocalcemia
and vit D def (Rickets)

* **Com** : NEC
Celiac in Diabetic pt $\bar{e}$ +ve
TTG and need for endoscopy
& biopsy

* **Develop** : motor GDD

* **clinical** : splenomegaly
Thyroid
Russel silver
Quadriplegia
diplegia
CMT

Egypt sept 2023 Day 2

*Com: speak to 20w pregnant about cleft lip and palat

*Com 2: NAI discovered in 3y
primary Ciliary dyskinesia nurse

*video 1. pertussis & tracheostomy &
NG Tube

2. Bell's palsy

*Hx: 13y epilepsy non Compliant

*Clinical: Thalassemia
ps MR
Quadriplegic CP
Cushing Syndrome

*Develop: 3.5y social & speech
Autistic

UK Feb 2023

* Com: Gastrostomy
DDH

* Video: Asthma
Cerebellar ataxia

* Hx: febrile neutropenia

* Develop: Cognitive

* Clinical: ps
abdomen ē scars and peg
spina bifida
NF1
General DM1 ē syringe pump
Hydrocephalus
Msk JIA

* Video: CHD
NAI

* Com: fecal impaction and soiling ē
no adherence to ttt

* **Com 2:** 13y fainted at school ē
iron def anemia

* **Hx:** spastic diplegia

* **Development:** 2.5y autism
assess social

Mumbai April 2025 1st day

Clinical: Resp CSLD

Cvs MR

abd HsM in thalassemia

Motor exam in L-L

Com: 1. NAI Counsel the mother

2. Counsel 15y old for H. pylori ttt

Hx: poor wt gain and anemia

* **Development**: Neuro regression - gross
motor assessment

* **Video**: Erb's palsy

stridor

Mumbai April 2025 2nd day

* Video: 1. rash in 5 months with
maculopapular rash and
peeling - SSSS

2. PDA in preterm

* clinical: CNS: L.L motor & knee
Contractures

Resp: CS LD

abd: thalassemia

CNS: MR & Cyanosis

* Development: speech delay
(social & cognitive)

* Hx: 1st episode of unprovoked seizure
GTCS in KCO febrile seizure
father worried about tumor

* Com: 1. RSV infection in hospital

2. preterm $\bar{e}$ NEC and TPN . Needs to be started on enteral feeds

Qatar day 1 April 2025

* Development: 2.5 y girl $\bar{e}$ down synd.

do gross motor

* History: 6 y boy $\bar{e}$ body & facial swelling

2nd time in 3 years

* Clinical: 1. 14 y boy $\bar{e}$ sternotomy scar
drain scar and Kocher scar

2. Msk of L.L in 2 y boy $\bar{e}$
spina bifida

3. Abd exam 10 y boy $\bar{e}$ HSM
small for age

4. Cranial nerve examination in
10 y $\bar{f}$ 3rd nerve palsy.

* Communication:

1. 20 wk pregnant with u/s showed bilat cleft lip and palat. Tell her about plans after delivery

2. Talk to junior trainee dr who altered notes of child admitted. Tell him about consequences

* video:

1. Bell's palsy

2. ITp

Pakistan April 2025 day 1

* Clinical: 1. Abd → acute hepatitis

2. Resp → Cystic fibrosis

3. Mps

4. spastic quadriplegic & malnutrition

* Hx: 4 y girl & recurrent Cough and wheeze (Asthma)

* Develop: Hgic stroke

* Com: 1. T.S Information giving

2. Suspected abuse (NAI)

* Video: 1. Nephrotic

2. SSSS

Pakistan April 2025 day 2

* Hx: vit K def → 6 wk went pale
in cot in midnight take focused
Hx.

* Develop: Shaken baby syndrome 2y old
baby in foster care
Fine motor assessment

* Clinical: 1. Abd: wilm's tumor
2. Resp: Asthma
3. General: Down $\bar{e}$ repaired
AVSD
4. Neuro: DMD vs Cp

* video: 1. Gilbert
2. pleural effusion

* Com: 1. DM type 1 Ttg +ve discuss
for celiac
Mgt diet plan
Next Confirmatory test

2. child ē burn, Mother was afraid of Cps to take her babies

Qatar April 2025 day 2

* **Com:**

1. NAI talk to mother about admission

2. T.S pt talk to mother about Confirmatory test

* **Video:**

1. SSSS

2. Nephrotic syndrome

* **Clinical:** 1. pt ē ptosis examine all cranial nerves

2. Cvs 6y stunted, dysmorphic median sternotomy & 2 drain scars. Pansystolic murmur ē thrill

3. Other: Mps short stature

4. abdomen : thalassemia

* **Development** :

preterm, NICU admission, hemiplegia

Motor assessment

* **History** : viral induced wheeze

Jeddah May 2025 day (1)

* **History** : Cephalhematoma

* **Com** : 1. Dermatitis Artifacts
2. delayed surgery CP pt

* **Uidea** : 1. Lymphadenitis
2. N.S

* **Develop** : Cong. hypothyroidism
do fine motor

* **clinical** : Dextrocardia
Residual ps from ToF
Cf, Quadriplegia
Paraplegia, Thalassemia
Wardenberg.

Jeddah May 2025 day 2 + UAE

* **History**: Kartagener syndrome

* **Com**:
1. Constipation & soiling
2. Cong hypothyroidism

* **video**: 1. stridor (laryngeal
hemangioma)

2. Herpes Zoster

* **Develop**: GDD for gross motor

Mostly Cong myopathy

or genetic causes

- Bilat SNHL, small head

squint, ptosis, +ve consanguinity

* Clinical: Thalassemia

Hemiplegia, Diplegia

Residual ps from Tof

prosthetic valve

Ehler danlos

Russel silver

Rickets

chronic Renal Failure

UAE May 2025 day 2

*History: orbital cellulitis 6y

lt eye swelling and redness

Social issue grandmother is sick

① From guidelines, Imp of vaccination

*Com: 1. MRSA : 21 days after

gastraschisis surgery shifted to

our unit to establish feeding

Main Concern : where to go home, hygiene

2. Teenager DM & albuminuria

he was smoker

*video: 1. ITP → child 3y & rash

2. portal HTN

* **Clinical:** 1. ps

2. Ch hemolytic anemia

3. cf e DIOS

4. Quadriplegia

* **Develop:** 18 m do Motor

Brain tumor at 3m on

anti epileptic, social issue

Egypt May 2025 day 1

* **History**: Ulcerative Colitis non Compliant

* **Video**: 1. per orbital cellulitis

2. CCHD

* **Com**: 1. vegetarian & iron def

2. Mom refuse discharge of

stable preterm

* **Develop**: Meningitis Cognitive

* **Clinical**:
Tof
DMD
Cp
Mps
Turner

Egypt May 2025 day 2

* **Clinical**: 1. Down operated CHD
ē residual murmur

2. Mosaic turner
3. splenomegaly for DD
4. Quadriplegic CP

* **Com**: 1. Anaphylaxis after sting
bite talk about epipen
2. Osteosarcoma, biopsy to confirm
parents don't want to tell child
ethical dilemma

* **History**: DM ē poor compliance as
shifted to another city

* **video**: 1. Epilepsy syndrome
2. ITP

* **Develop**: GDD ē Autistic Features
do fine motor.

Singapore May 2025 day 1

*History: day time wetting

*video: 1. Laryngeomalacia

2. CHD

*Comm: 1. HIV withholding info.

2. Febrile Convulsions

*Develop: BWS do Cognitive

*clinical: Job syndrome

Residual murmur VSD

ps after median sternotomy

Hemiplegia ē facial drop

Thalassemia.

Sri Lanka May 2025 day 1

Hx : Dermatomyositis

Development : Meningitis , Cognitive assessment

Com : 1. Apology hypoglycemia
2. Biliary atresia

video : 1. Nephrotic
2. viral induced wheeze

Sri Lanka May 2025 day 2

* Development : Global delay ē hearing aid , assess Motor
* Congenital rubella syndrome

* HX : oligoarticular JIA

* Com : 1. Eczema, Mom refuse to follow ē gp

2. H. pylori

* video: Croup

Acute viral hepatitis

* clinical: - VSD

- thalassemia

- Hemiplegia

- Neurofibromatosis

UK June 2025 1st day

* **Hx**: Down & Leukemia

* **Develop**: NAI with hemiplegia

For motor assessment

* **Videos**: 1. DDH

2. Cushing syndrome

* **Com**: 1. wrong Lp

2. adolescent drink alcohol

* **Clinical**: different cases

UK 13 June 2025

* Hx: Known case of dermatomyositis
came ē exacerbation.

* Com: 1. NAI

2. Infected eczema for follow up

* video: 1. pneumonia

2. osteomyelitis

* Develop: known case of phenyl -
ketonuria for Cognitive
assessment.

* Clinical: 1. Median sternotomy,
webbed neck, 4/6 pan systolic murmur
2. DMD on wheelchair

3. 17 years, mass in Lt iliac fossa

ē Large scar → post renal transplant

4. child ē tracheostomy tube, Large

tongue, hypopigmented hair and eye

brows, chest → crackles

Jeddah sept 2025 day 1

* Clinical :

Cystic Fibrosis

DMD

Quadriplegia

operated CHD

Pacemaker

Thalassemia

short stature for DD

* Video :

- viral induced wheeze

- Infantile spasm

* Com :

- laryngomalacia to mother

- Formula given to exclusive

breast feeding without telling

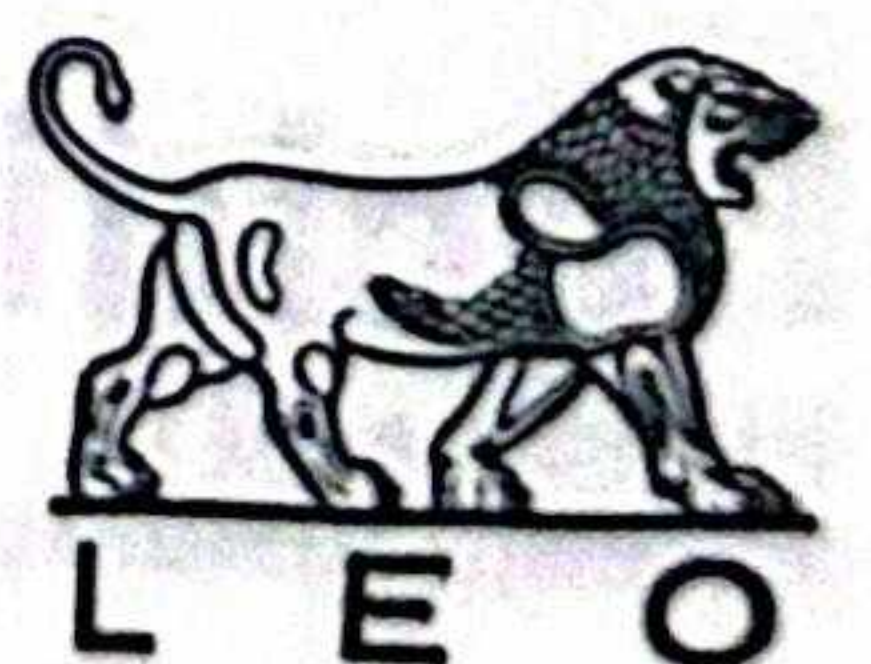
the mother.

* History :

- Constipation.

Fucidin®

Fusidic acid /
sodium fusidate



* **Development:** Cognitive assessment
in Mps & regression &
Autism.

Jeddah sept 2025 day 2

* **History:** orbital cellulitis

* **video:** - Croup

- PDA

* **Com:** - peanut allergy

- DDH

* **Development:** Fits at age of
3 months, on Na valproate

Motor assessment

* **clinical** : - Ehler danlos

- hemiplegia

- DMD

- Ch. supp. lung drs.

- HsM

- Residual ps



Chennai sept 2025 day 1

* Video: - pDA

- Croup

* Com: 1. peanut allergy : given peanut butter by staff, subsided & antihistamin was admitted for viral wheeze which also improved & back to back nebs.

2. DDH diagnosed at 8 months

Mother was angry from delayed diagnosis

* History: Acute onset of eye swelling not able to open affected eye.

* Develop: 2y & spasmodic move at 3m followed by developmental delay & regression.

* **Clinical**: 1. Med sternotomy in 6 y

ē clubbing No cyanosis

2. Abd: roof top scar ē

hepatosplenomegaly

3. CNS: quadriplegic cp

4. Others: Mps

Chennai sept 2025 day 2

* **Video**: 1. shingles

2. Facial palsy

* **Com**: 1. Rop for Laser in pre term

2. Teenage ē thalassemia

* **History**: known asthma ē poor control



* **Development**: sturge weber ē GDD

for Cognitive assessment

Egypt sept day 1 2025

* **History**: CF ē Cough and green sputum
(pseudomonas)

* **Develop**: post brain tumor ē
hemiplegia . Assess fine
motor

* **Video**: 1. shingles in 6 months
baby on steroid.

2. Celiac



* **Com**:

1. CoA with diff BP in

UL & LL and Fainting

→ Some say As: Fainting ē weak peripheral pulsation.

2. Glycopyrolate in Cp

* **Clinical**: + Quadriplegia

* Noonan syndrome

* Ch. hemolytic anemia

* operated heart ē

residual ps.

* ps in Williams.

Fucidin®

Fusidic acid /
sodium fusidate



Egypt sept 2025 day 2

*History: Limping child for DD
(septic arthritis)

*Com: 1. Talk ē grandmother

Rheumatologist. preterm 29wks 1.5kg
was on CPAP then deteriorated then
on ventilator, recieved surfactant
went through preterm Comp. why MV
why deteriorating till f/4

2. Nurse while taking sample
the nurse got needle prick. Girl came
from endemic area (HIV). She had before
fever and was sick. Task to sit ē mom



to take her Consent to Collect blood
from her daughter and know if she has
any disease

* **video**: 1. pertussis
2. Infantile spasm

* **Clinical**: 1. systolic murmur in
non operated child.

2. spina bifida

3. Marfan

4. Mps Abd & general ex

* **Develop**: Hydrocephalus, shunt
obstructed labour, Fever before
→ speech & social assessment



Singapore oct 2025 day 1

* **History**: Bronchiolitis in 14 months old boy.

* **Develop**: BWS for Cognitive ass.

* **video**: 1. pleural effusion
2. Leukemia

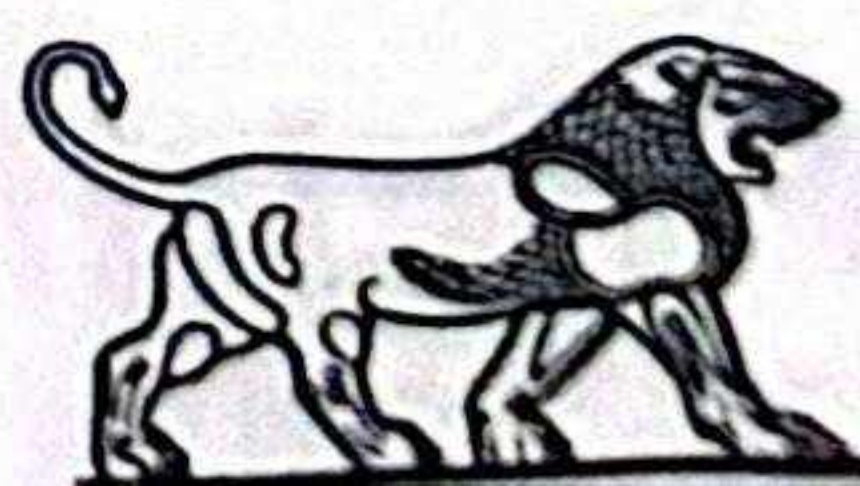
* **Com**: 1. Chapron
2. pVL

* **Clinical**: VSD

Dextrocardia

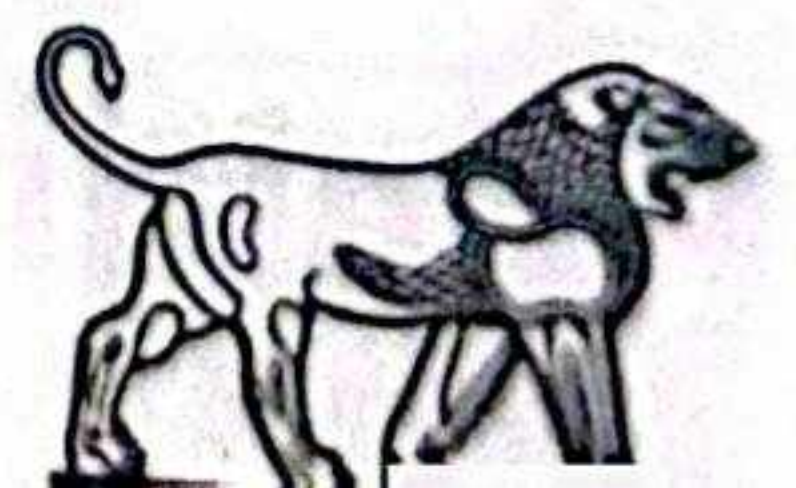
pallor & splenomegaly for DD

Hemiplegia



UK 12 Oct 2025

- * History: chest pain
- * Develop: GDD for fine motor assess.
- * Com:
 - 1. Nephrotic syndrome relapse
 - 2. Aminophylline overdose
- * Video:
 - 1. ITP
 - 2. Transient synovitis.
- * Clinical:
 - 1. eye exam → ptosis
 - 2. Msk: arthritis / Ehler / Marfan
 - 3. CVS: ps / innocent murmur
 - 4. Abd: post Kasai scar



UK 13 oct 2025

* Com:

1. anorexia nervosa in 13 y girl
speak to mother and explain
admission

2. Eczema in 9 months has history of
eczema herpeticum before. She was
referred now from dermatologist
because of severe attacks of eczema
started on new milk formula

* video: 1. acute mastoiditis

2. pyloric stenosis

* Develop: Down on GDD
Assess fine motor.

* History: Breath holding attack
2y old fell down while playing
turned blue and unresponsive

* **Clinical**: • CVS → ESM

- Eye exam in Horner syndrome
- Abd: 14 y old girl ē Rt

Subcostal scar and Hx of neonatal Jaundice (CBA)

- Marfan syn → DGAIS
- Case ē chest deformity
- Chest exam child 4 years small

for age with large head and macroglossia with skeletal dysplasia, peg tube & squint

→ Discussion: Causes and mgt

- Neuro → hemiplegia

Kolkata Nov day 1 2025

* **History**: Functional Constipation

* **Develop**: Gross motor in DMD

* **Video**: 1. HIE

2. Croup

* **Com**: 1. Leukemia breaking bad news
• pancytopenia & Hsmegaly
Counsel for BM biopsy

2. antenatal renal pelvis dilatation
16mm for post natal Counselling
and investigation.
• U/s after 1wk and prophylactic
antibiotics

* **Clinical**: 1. DMD
2. ps
3. post Kasai (HSM + rooftop)
4. JiA / hemarthrosis
5. USD

UAE NOV 2025 day 1

* **History** → anaphylaxis in a known

Case of food allergy

* **Com**: 1. Obesity to medical student

2. Rolandic epilepsy to mother

* **Develop**: speech delay in a child

ē small private parts and

undescended testis, did operation

and fixed.

* **Video**: 1. vesicular rash on flexors

ē 2ry infection (Impetigo ??)

2. DDH



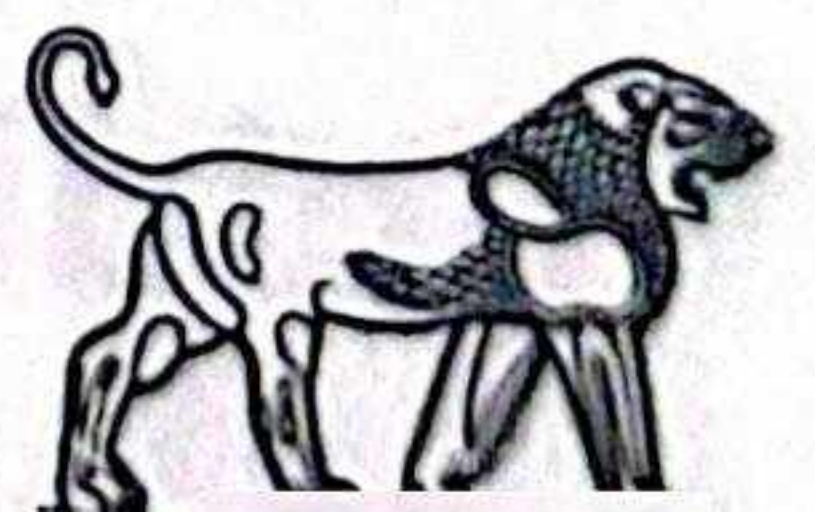
* **Clinical** : Noonan
ataxia
Hemiplegia
Williams
AS
Rickets
Thalassemia
Kartagener
Down $\bar{e}$ VSD

UAE NOV 2025 day 2

* **History** : Sandifier

5y, post herpetic encephalitis
at age of 2 on antiseizure. Came
 $\bar{e}$ generalized stiffness after feeding.

* **Com** : 1. Global develop delay
in 11 months with microcephaly



2. Mother ē depression ē 4 day old

Newborn ē Jaundice . speak to her

about admission & photo therapy

* **Develop** : 2y assess fine motor

went down ē milestones to 3-6 m

Global delay ē Cataract, Low set ears

& hypotonia.

* **Video** : 1. Eczema herpeticum

2. Antalgic gait →

osteomyelitis

* **Clinical** :

Mps

Thalassemia

Hemiplegia

CF

spina bifida.

Jeddah NOV 2025 day 1

* video: 1. intestinal obstruction

child has vomiting, video showing
baby ē abdominal distension.

2. Hsp

* Com: 1. Talk to medical student

Mother is working and baby spent all
time ē grandmother. It was about

challenging behavior & why not ADHD

2. Baby 10 wk, fever, irritability
abnormal urine test. It was about UTI
work up & images

* **History**: shortness of breathing while doing shopping suddenly after eating sweets. It was anaphylaxis and no epipen, plan or follow up. Has history of Cow milk allergy. Family Hx of allergy and asthma. All discussion and advices for anaphylaxis

* **Development**: 2.5 y assess Cognitive Empty Hx, Autism details

* **Clinical**:
Thalassemia
DMD
CHD ē residual murmur
short stature
Russel silver
Dextrocardia



Jeddah Nov day 2 2025

*Com: 1. Vomiting in infant, normal growth, GERD, then DD of vomiting in this age went through Cong pyloric stenosis

2. Cf 9 year pseudomonas infection. Talk to father for mgt plan is it my fault as we went pool. Can he travel France ē us

*Clinical: Klippel Feil Msk

- Median sternotomy ē residual p

- Quadriplegia clear clonus

- thalassemia

- Kartagener

- Dextrocardia

* History: viral myocarditis

* Development: 18 months & brain

tumor at age of 3 months

Motor assessment

* video: 1. High step gait → GBs

2. scarlet fever.

Egypt Dec 2025 day 1

* **History**: Crohn's

* **Develop**: Regression & Autism

* **clinical**: Achondroplasia

CHD ē residual murmur

Thalassemia

Cp

* **video**: 1. squint : 6th N palsy

2. viral induced wheeze

* **Com**: 1. obesity

2. chronic fatigue syn.

Egypt Dec 2025 day 2

* **clinical**: operated VSD & murmur

Achondroplasia

spherocytosis

Thalassemia, Cp

* **History**: Hematuria, Mother worried about bleeding per rectum.

* **Com**: 1. Breastfeeding midwife upset of possible admission, we'll take labs from a baby losing more than 10% of his wt and jaundice
2. 15 y Cf refuse admission to ward for IV antibiotics, smoker.

* video: pneumonia

self harm

* Development: Fine motor in
william syndrome